

Recurrent genital herpes

Indications for therapy

Genital herpes recurrences are self-limiting and generally cause minor symptoms. The level of distress and the disruption to an individual's sexual and social life is often independent of the frequency of symptoms. Decisions about managing clinical recurrences should be made in partnership with the patient. Management strategies include supportive therapy only, episodic antiviral treatment, and suppressive antiviral therapy. The most appropriate strategy may vary over time depending on recurrence frequency, symptom severity, and relationship status. For most patients, supportive management is sufficient, with simple local measures such as saline baths or topical petroleum jelly being adequate.

Episodic antiviral treatment

Oral aciclovir, valaciclovir, and famciclovir are effective in reducing the duration and severity of recurrent genital herpes, with a median reduction in healing time of 1–2 days. Head-to-head studies show no significant advantage of one agent over another, nor of extended 5-day regimens over ultra-short courses. Prodrugs (valaciclovir, famciclovir) offer simplified twice-daily dosing. Treatment initiated within 24 hours of symptom onset is most effective, with lesion abortion reported in up to one-third of patients when treatment is started early. To ensure prompt initiation, patients should keep a supply of medication available at all times and not wait to obtain it after symptoms begin.

Initial short-course regimens:

- Aciclovir 800 mg three times daily for 2 days
- Famciclovir 1 g twice daily for 1 day

- Valaciclovir 500 mg twice daily for 3 days

Alternative 5-day regimens:

- Aciclovir 400 mg three times daily
- Aciclovir 200 mg five times daily
- Valaciclovir 500 mg twice daily
- Famciclovir 125 mg twice daily

Suppressive therapy

Early trials of suppressive therapy focused on patients with ≥ 6 recurrences per year, but more recent studies include those with milder disease, showing benefit across all recurrence frequencies. The decision to initiate suppressive therapy is individualized, balancing recurrence frequency, personal impact, transmission concerns, and treatment burden.

Most patients experience a substantial reduction in recurrence frequency on suppressive antiviral therapy, although occasional symptomatic episodes may still occur.

Aciclovir has the most extensive long-term safety data (over 18 years of surveillance), with no evidence of cumulative toxicity or organ damage. Dose adjustment is required only in severe renal impairment. Routine blood monitoring is not recommended in otherwise healthy individuals. Periodic reassessment of the need for ongoing therapy is advisable, as patient circumstances may change. However, many patients do not experience significant changes in disease pattern after discontinuation, even following prolonged suppression.

A small number of immunocompetent patients continue to have frequent, virologically confirmed recurrences despite full-dose antiviral therapy. HSV resistance is unlikely in this group. Further immunological investigations are not useful beyond HIV testing. Interferon therapy is not recommended due to lack of robust evidence and limited availability.

Recommended regimens

The optimal total daily dose for suppressive aciclovir therapy is 800 mg.